HOUSE OF REFUGE REHABILITATION CENTRE

OFFICIAL TREATMENT PROTOCOL

Evidence-Based | Christ-Centered | Trauma-Informed | Reintegration-Focused

A Freedom Foundation Programme

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> *"He has sent me to proclaim freedom for the prisoners and recovery of sight for the blind, to set the oppressed free..." - Luke 4:18*

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1. PROGRAMME OVERVIEW & PHILOSOPHY

1.0 Global Evidence Base

This Treatment Protocol is built on six internationally recognised frameworks and adapted for the Nigerian clinical and cultural context:

1. NIDA's 13 Principles of Effective Drug Addiction Treatment (National Institute on Drug Abuse, USA) The foundational research-based guide established by NIDA after 20+ years of addiction treatment research. HOR's protocol aligns with all 13 principles, notably: no single treatment is appropriate for everyone; treatment needs to be readily available; effective treatment attends to multiple needs of the individual (not just drug abuse); an individual's treatment plan must be assessed continually and modified; treatment does not need to be voluntary to be effective (hence our Outpatient Engagement Pathway); and remaining in treatment for an adequate period of time is critical (our minimum 12-week / 84-day standard, based on evidence that outcomes improve significantly beyond 90 days).

Source: NIDA, Principles of Drug Addiction Treatment: A Research-Based Guide, Third Edition (nida.nih.gov)

2. WHO Mental Health Gap Action Programme (mhGAP) Guidelines (2023 Update) The WHO mhGAP intervention guide provides evidence-based recommendations for substance use disorder treatment in low- and middle-income countries. HOR implements mhGAP recommendations for: psychosocial interventions as first-line treatment, structured withdrawal management, and integration of mental health screening (PHQ-9, GAD-7) into substance abuse treatment. The mhGAP framework specifically validates our approach of training non-specialist health workers to deliver evidence-based interventions under clinical supervision.

Source: WHO mhGAP Guideline for Mental, Neurological and Substance Use Disorders, 2023 (who.int)

3. UNODC TREATNET Quality Standards (United Nations Office on Drugs and Crime) UNODC's TREATNET training package, developed by an international consortium led by UCLA, sets quality standards for drug dependence treatment in developing countries. Over 10,000 practitioners in 20+ countries have been trained using this framework. HOR's protocol incorporates TREATNET standards for: comprehensive assessment, individualised

treatment planning, evidence-based interventions (CBT, MI), community reintegration, and structured aftercare follow-up.

Source: UNODC TREATNET Training Package (unodc.org/treatment-and-care)

4. ASAM Criteria, Fourth Edition (2024) (American Society of Addiction Medicine) The most widely used comprehensive set of standards for addiction treatment placement. ASAM's six-dimensional assessment framework informs HOR's admission criteria and treatment matching: (1) Acute Intoxication/Withdrawal Potential, (2) Biomedical Conditions, (3) Emotional/Behavioral/Cognitive Conditions, (4) Readiness to Change, (5) Relapse/Continued Use Potential, (6) Recovery/Living Environment. HOR operates at ASAM Level 3.5 (Clinically Managed High-Intensity Residential Services).

Source: ASAM Criteria Fourth Edition, 2024 (asam.org)

5. Therapeutic Community (TC) Model (De Leon, Daytop) The Therapeutic Community approach, pioneered by Dr. George De Leon at Daytop Village, provides the residential community framework for HOR. Research demonstrates that TC participants who stay 90+ days show 9x better outcomes than those who leave early, and 88% of 2-year completers are employed and crime-free. HOR integrates TC principles of "community as method": the therapeutic community itself is the primary instrument of change, through structured daily routines, peer accountability, hierarchical work roles, and group responsibility.

Source: De Leon, G. "The Therapeutic Community," PMC 2020; Daytop Village Outcomes Research

6. Teen Challenge Outcomes Research (Faith-Based Integration) Adult & Teen Challenge, the world's largest faith-based residential recovery network (215+ centres), provides the evidence base for Christ-centred recovery programming. Studies show 74% of graduates report no substance use 6 months post-programme, and 87.5% of former marijuana abusers were abstaining 7 years after completion. Research identifies four key elements contributing to recovery: programme structure, relationships with others, experiences of God, and individual initiative. HOR's spiritual formation protocol is modelled on this evidence while maintaining clinical rigour.

Source: Wilder Research Study of Teen Challenge (2009); Dr. Catherine Hess Study; ACADC Research Review

How These Frameworks Integrate at HOR

Framework	HOR Application

NIDA 13 Principles	Programme design, individualised treatment, duration, multiple needs
WHO mhGAP	Assessment instruments, psychosocial interventions, LMIC adaptation
UNODC TREATNET	Quality standards, staff training, community reintegration
ASAM Criteria 4th Ed	Admission criteria, level of care determination, 6-dimensional assessment
TC Model (De Leon)	Residential community structure, peer accountability, daily routine
Teen Challenge	Spiritual formation integration, faith-based outcome evidence

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1.1 Programme Identity

House of Refuge (HOR) is a 24-bed residential drug and alcohol rehabilitation centre located in Lekki, Lagos, Nigeria. Established in 2006 by Freedom Foundation, HOR operates at the intersection of clinical excellence and Christian compassion.

Capacity: 24 beds across 4 wings (configurable Male/Female ratio, default 3:1) **Target Population:** Adults aged 18-65 with moderate-to-severe Substance Use Disorder **Programme Duration:** 12 weeks (84 days), extendable to 26 weeks where clinically indicated **Regulatory Registration:** Lagos State Ministry of Health, Lagos State Ministry of Youth, Sport and Social Welfare, NDLEA

1.2 Treatment Philosophy

HOR's treatment philosophy rests on four integrated pillars:

1. **Evidence-Based Clinical Practice** - Cognitive Behavioural Therapy (CBT), Motivational Interviewing (MI), and structured Relapse Prevention form the therapeutic backbone. All interventions are grounded in internationally validated approaches with proven efficacy for Substance Use Disorders.

2. **Christ-Centered Spiritual Formation** - The Gospel of Jesus Christ is the ultimate foundation of healing and transformation. Clinical treatment and spiritual formation are not competing priorities - they are complementary pillars of the same healing journey.

3. **Trauma-Informed Care** - Addiction is understood as often rooted in unresolved trauma. All staff interactions are guided by safety, trustworthiness, collaboration, empowerment, and cultural sensitivity.

4. **Reintegration-Focused Outcomes** - Successful rehabilitation is measured not by programme completion but by the client's sustainable, productive, God-honouring reintegration into family, church, and community.

1.3 Core Values - FREE

Value	Application
F - Freedom	True freedom from addiction, shame, and fear is available through Christ and evidence-based recovery
R - Responsibility	We model and teach responsibility as a cornerstone of recovery and mature Christian character
E - Exemplary Leadership	Every staff member leads by example in godliness, professionalism, compassion, and integrity
E - Empathy	We see every client through the eyes of Christ with unconditional positive regard

1.4 Guiding Principles

- Person-Centred Care: Every client receives an individualised treatment plan
- Dignified, Non-Stigmatising Care: No client is defined by their addiction
- Family and Community Integration: Recovery does not happen in isolation
- Continuous Quality Improvement: Protocol reviewed annually against emerging evidence

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2. ADMISSION PROTOCOL

2.1 The Willingness Criterion - Primary Gateway

NON-NEGOTIABLE: Admission to residential care requires demonstrated, voluntary willingness to participate in recovery.

Willingness is assessed clinically using:

- URICA (University of Rhode Island Change Assessment) Stage of Change evaluation
- Motivational Interviewing-based dialogue
- Clinical interview by Program Director (Medical Doctor)

Unwilling clients are NOT turned away. They are enrolled in the Outpatient Engagement Pathway (Section 2.5).

2.2 Beneficiary Profile

Criterion	Specification
Gender	Male & Female
Age Range	18 to 65 years
Primary Condition	Substance Use Disorder (moderate to severe) - alcohol, cannabis, opioids, stimulants, inhalants, or polysubstance
Psychiatric Status	Stable or manageable. No active psychosis, no acute suicidal/homicidal risk
Medical Status	Medically stable or manageable within HOR's 24/7 nursing scope
Willingness	Voluntarily seeking treatment; demonstrates insight and readiness

2.3 Five Inclusion Criteria (ALL Must Be Met)

#	Criterion	Assessment Method
1	**Willingness** - voluntary readiness to engage	URICA, MI-based dialogue, clinical interview
2	**SUD Diagnosis** - meets DSM-5 criteria for moderate-to-severe SUD	ASI (Addiction Severity Index), ASSIST screening
3	**Medical Stability** - manageable within HOR's scope	Program Director medical evaluation
4	**Functional Capacity** - can participate in group therapy and counselling	Clinical interview, cognitive screening if indicated
5	**No Immediate Risk** - no active suicidal ideation with intent/plan, no homicidal intent, no acute psychosis	Standardised safety screening, C-SSRS

2.4 Exclusion Criteria (Immediate Referral Required)

Condition	Action

Active Psychosis	Referral to Yaba Psychiatric Hospital. HOR door remains open upon stabilisation
Severe Suicidal/Homicidal Risk	Emergency referral to hospital A&E. Family notified immediately
Drug-Induced Psychosis (DIP)	Referral for psychiatric stabilisation. Re-assessment upon resolution
Antipsychotic Medication Need	Referral to psychiatrist. Re-admission when pharmacologically stable
Severe Cognitive Impairment	Referral to neurological/cognitive rehabilitation specialist
Active Legal Detention	Liaise with legal authority. Offer outpatient engagement
Children/Family Co-habitation	Refer to family-inclusive outpatient services

Golden Rule: Every excluded client leaves with: (1) a written referral, (2) a compassionate conversation, (3) HOR's contact number, (4) basic harm reduction information. Follow-up within 72 hours.

2.5 Outpatient Engagement Pathway (Non-Ready Clients)

Component	Description
Motivational Interviewing	Weekly 45-60 min individual sessions using MI techniques
Psychoeducation	Addiction as a brain disorder, recovery process education
Family Counselling	With consent - psychoeducation about enabling vs supporting
Harm Reduction Guidance	Practical safety information to reduce immediate risk
Spiritual Engagement	Voluntary invitation to prayer, pastoral conversation, chapel
Reassessment	Every 2-4 weeks via URICA and clinical interview

2.6 Dual Admission Pathways

Pathway A - Family-Supported Admission:

- Family consent and active engagement documented
- Financial agreement signed (payment plan available)

- Primary Family Support Person (PFSP) identified and named
- Family participates in minimum 2 therapy sessions + 1 pre-discharge meeting
- Family completes psychoeducation on addiction and enabling behaviour
- Home environment assessed for safety and sobriety-supportiveness

Pathway B - Community-Supported Admission:

- Vulnerability and Needs Assessment (VNA) by Social Worker
- Sponsorship, community funding, or HOR Compassion Fund confirmed
- Social Worker assigned as primary case manager (treatment + 12-month aftercare)
- Church/faith community partner identified
- Supported housing referral initiated by Week 8
- Vocational assessment completed before discharge

2.7 Eight-Step Admission Process

Step	Content	Responsible
1. Initial Contact	Compassionate engagement; explain programme, expectations, costs	Program Officer / Counsellor
2. Pre-Screening	ASI, ASSIST, URICA administered. Willingness assessed. Eligibility determined	Program Director
3. Clinical Assessment	Full medical evaluation. Psychiatric screening. Psychosocial assessment. Medical tests ordered	Doctor + Nursing Staff
4. Admission Decision	Admit / Refer / Defer decision documented. Pathway activated	Program Director
5. Documentation	7 consent forms signed (Admission Agreement, Informed Consent, Detox Consent, Confidentiality, Family Confidentiality, Belongings Receipt, Financial Agreement, Rights Charter)	Program Officer + Client
6. Intake & Orientation	Property inspection. Contraband check. Belongings inventory. Bed assignment. Full orientation	House Master + Staff
7. Treatment Planning	Columbia Model treatment plan within 72 hours. Client co-authors goals	Clinical Lead + Counsellor
8. Detoxification	Medical detox under doctor supervision with 24/7 nursing (Weeks 1-2)	Doctor + Nursing Staff

2.8 Pre-Admission Medical Tests (Mandatory within 48 hours)

Test	Clinical Rationale
HIV/AIDS Screening	Baseline status; HIV-positive clients NOT excluded, linked to ART
Hepatitis A & B Serology	Co-morbidity assessment; immunisation needs
Urine Drug Screen (UDS)	Confirms substances of use; baseline for detox planning
Full Blood Count (FBC)	Identifies anaemia, infection, haematological issues
Liver Function Tests (LFTs)	Critical for alcohol-dependent clients; medication safety
Malaria Parasite Smear	Rules out malaria as confounding factor in withdrawal presentation
Widal Test	Rules out typhoid fever (Nigerian clinical context)
Chest X-Ray (CXR)	Screens for pulmonary TB, aspiration pneumonia
Blood Glucose (Random)	Identifies diabetes/hypoglycaemia for nutritional management

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3. CLINICAL ASSESSMENT FRAMEWORK

3.1 Assessment Instruments

Instrument	Purpose	Timing
ASI (Addiction Severity Index)	Comprehensive addiction assessment across 7 domains	Admission
ASSIST (Alcohol, Smoking and Substance Involvement Screening Test)	Substance-specific risk screening	Admission
URICA (University of Rhode Island Change Assessment)	Stage of change / readiness assessment	Admission, bi-weekly
C-SSRS (Columbia Suicide Severity Rating Scale)	Suicide risk screening (6 items)	Admission, weekly, any concern

CIWA-Ar (Clinical Institute Withdrawal Assessment - Alcohol)	Alcohol withdrawal severity (10 items, max 67)	Detox phase, scheduled rounds
COWS (Clinical Opiate Withdrawal Scale)	Opioid withdrawal severity (11 items, max 47+)	Detox phase, scheduled rounds
AUDIT (Alcohol Use Disorders Identification Test)	Alcohol use severity (10 items, max 40)	Admission + Discharge
DAST-10 (Drug Abuse Screening Test)	Drug abuse severity (10 items, max 10)	Admission + Discharge
PHQ-9 (Patient Health Questionnaire)	Depression screening (9 items, max 27)	Admission + Discharge
GAD-7 (Generalised Anxiety Disorder)	Anxiety screening (7 items, max 21)	Admission + Discharge

3.2 CIWA-Ar Scoring Protocol (Alcohol Withdrawal)

10 assessment items: Nausea/vomiting, Tremor, Paroxysmal sweats, Anxiety, Agitation, Tactile disturbances, Auditory disturbances, Visual disturbances, Headache, Orientation/clouding

Score Range	Severity	Clinical Action
0-8	Mild	Continue monitoring q4h. No medication indicated
8-15	Moderate	Start withdrawal medication. Monitor q2h
15-20	Severe	Symptom-triggered benzodiazepine protocol. Monitor q1h
>20	Very Severe / DT Risk	Intensive monitoring. Consider transfer if not stabilising

3.3 COWS Scoring Protocol (Opioid Withdrawal)

11 assessment items: Resting pulse rate, Sweating, Restlessness, Pupil size, Bone/joint aches, Runny nose/tearing, GI upset, Tremor, Yawning, Anxiety/irritability, Gooseflesh skin

Score Range	Severity	Clinical Action
0-5	Minimal	Continue supportive care

5-12	Mild	Supportive care, comfort medications
13-24	Moderate	Monitor closely, symptom management
25-36	Moderately Severe	Active pharmacological management
>36	Severe	Intensive management, consider transfer

3.4 C-SSRS Risk Assessment Protocol

6 screening questions (all Yes/No):

1. Wish to be dead
2. Suicidal thoughts (non-specific)
3. Suicidal thoughts with method (without plan)
4. Suicidal intent (without specific plan)
5. Suicidal intent with plan
6. Suicidal behaviour (with timeframe: lifetime / past 3 months / past month / since last visit)

Additional safety screening:

- Active psychosis indicators (select from standard list)
- Homicidal ideation (Yes/No)
- Self-harm history (None / Historical / Past 3 months / Current)

Risk Level	Criteria	Required Action
Low	Q1-2 only, no other flags	Continue routine monitoring
Moderate	Q3-4, or past self-harm	Increase observation, notify Clinical Lead, document
High	Q5, or Q3-4 with self-harm history	Immediate 1-to-1 supervision, notify Program Director, contact family
Critical	Q6 (recent), or active psychosis/homicidal ideation	Emergency protocol: refer to Yaba Psychiatric Hospital or nearest A&E

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4. TREATMENT MODEL: 4-PHASE / 12-WEEK PROGRAMME

4.1 Phase Overview

Phase	Weeks	Focus	Key Milestones
Phase 1: Medical Stabilisation	1-2	Detoxification, vitals monitoring, nutritional recovery, orientation	Medical detox complete, baseline assessments complete, treatment plan initiated
Phase 2: Therapeutic Foundation	3-6	CBT, relapse prevention, psychoeducation, individual counselling, spiritual formation begins, life skills begin	Week 4 MDT treatment plan review, triggers identified, coping skills developing
Phase 3: Deepening & Skills Building	7-10	Advanced therapy, trauma processing, family systems, vocational assessment, PRPP development, evangelism	Week 8 MDT review, PRPP drafted, vocational pathway identified, family therapy sessions complete
Phase 4: Reintegration & Graduation	11-12	Pre-discharge assessment, family/community meeting, aftercare plan, graduation ceremony, Alumni Programme enrolment	All 6 graduation criteria met, PRPP finalised, church placement confirmed, aftercare plan signed

4.2 Daily Schedule (5:30 AM - 11:00 PM)

Time	Activity	Responsible
5:30 AM	Wake-up Call & Personal Hygiene	House Master
6:00 AM	Morning Exercise / Boot Camp (45 min)	House Master
6:45 AM	Personal Shower & Grooming	Residents

7:00 AM	Morning Prayer & Devotions (Praise, Worship, Prayer, Word)	Chaplain / House Master
8:00 AM	Breakfast	Cook / Domestic Staff
8:30 AM	Morning Community Meeting (accountability, day preview)	House Master / Counsellor
9:00 AM	Bible School / Spiritual Formation Class (2 hours)	Chaplain / Bible Teachers
11:00 AM	Group Therapy - CBT / Relapse Prevention / Psychoeducation (rotated)	Clinical Lead / Counsellor
12:30 PM	Lunch	Cook / Domestic Staff
1:30 PM	Life Skills Training Group (rotating modules)	Counsellor / Social Worker / Nurse
2:30 PM	Work & Craft / Vocational Skills / Creative Therapies	House Master / Counsellor
3:30 PM	Individual Counselling Appointments (scheduled)	Assigned Counsellor
4:00 PM	Physical Activity / Sports / Gym / Recreation	House Master
5:00 PM	Personal Time - prayer, Bible study, journaling (supervised)	House Master
6:00 PM	Evening Meal / Dinner	Cook / Domestic Staff
7:00 PM	Evening Chapel / Bible Study / Testimony / Discipleship Group	Chaplain / Counsellor
8:30 PM	Wrap-Up Meeting (day review, reflection, accountability, prayer)	House Master / Counsellor
9:00 PM	Personal Time - journaling, reading, fellowship (supervised)	House Master
10:30 PM	Evening Medication Round (if applicable)	Nurse on Duty

11:00 PM	Lights Out - all rooms	House Master

Schedule Notes:

- Nursing rounds: 6:00 AM, 12:00 PM, 6:00 PM, 10:30 PM daily
- Sunday: Modified for Chapel Service (9:00-11:30 AM). Afternoon free with family visits (12:00-6:00 PM)
- Weeks 1-2 (Detox): Modified to reduce therapeutic demands. Rest, nutrition, and medical monitoring priority
- Fasting: Permitted with doctor approval. NEVER during detox or withdrawal

4.3 Weekly Therapy Schedule

Component	Frequency	Duration	Responsible
Individual CBT	1x weekly	60 min	Assigned Counsellor
Group CBT / Relapse Prevention	3x weekly	90 min each	Clinical Lead / Counsellor
Psychoeducation Group	2x weekly	60 min	Nurse / Counsellor
Life Skills Group	3x weekly	60 min (rotating modules)	Counsellor / Social Worker
Spiritual Formation Class	5x weekly (Mon-Fri)	120 min	Chaplain
Sunday Chapel Service	Weekly	150 min	Chaplain + Guest Speakers
Evening Bible Study / Discipleship	Daily	90 min	Chaplain / House Master
Family Therapy (Pathway A)	Min 2 sessions + 1 pre-discharge	60-90 min	Clinical Lead / Counsellor
Community Group Work (Pathway B)	Weekly	60 min	Social Worker
Peer Recovery Circle	Daily (informal, evening)	30 min	Peer-facilitated, staff-supervised
Medical / Nursing Review	Daily + weekly doctor	Varies	Doctor + Nursing Staff

Relaxation Group	Weekly (Friday PM)	60 min	Nurse / Counsellor

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5. DETOXIFICATION PROTOCOL

5.1 Scope

Medical detoxification is conducted under physician supervision with 24/7 nursing monitoring during Weeks 1-2. The goal is safe, medically managed withdrawal with symptom relief, not punishment.

5.2 Alcohol Detox Protocol

- **Assessment:** CIWA-Ar scoring at admission, then q4h (q2h if score >15, q1h if >20)
- **Medication:** Symptom-triggered benzodiazepine protocol (Chlordiazepoxide or Diazepam)
- **Nutritional Support:** Thiamine 100mg IM/IV x 3-5 days, then oral. Multivitamin. Hydration monitoring
- **Monitoring:** Vital signs q4h minimum. Seizure precautions. DT observation protocol
- **Duration:** Typically 5-7 days. Extended monitoring as clinically indicated
- **Completion Criteria:** CIWA-Ar < 8 for 24 hours. Vital signs stable. No seizure risk

5.3 Opioid Detox Protocol

- **Assessment:** COWS scoring at admission, then per clinical schedule
- **Medication:** Supportive symptom management (Loperamide for GI, Clonidine for autonomic symptoms, NSAIDs for pain, Promethazine for nausea). NO opioid agonist therapy at HOR
- **Monitoring:** Vital signs. Hydration. Psychological support
- **Duration:** Typically 7-14 days depending on substance and duration of use
- **Completion Criteria:** COWS < 5 for 24 hours. Vital signs stable. Adequate oral intake

5.4 Cannabis / Stimulant Detox Protocol

- **Assessment:** Clinical observation. No validated withdrawal scale required
- **Monitoring:** Mood, sleep, appetite, agitation levels
- **Support:** Symptomatic management (sleep hygiene, anxiolytics if indicated, nutritional support)

- **Duration:** Variable. Typically 3-7 days for acute symptoms

5.5 Medication Management Standards

- **No opioid-containing medications** permitted in the facility under any circumstances
- All medications surrendered to nursing staff on admission
- All medications administered exclusively by licensed nursing staff
- Documented in Medication Administration Record (MAR) at each round
- Medication storage: locked cabinet, controlled access, stock reconciliation weekly
- No resident retains any medication (exceptions: nitroglycerin, insulin, seizure medication, asthma medication - doctor-approved only)

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6. THERAPEUTIC INTERVENTIONS

6.1 Treatment Plan Structure (Columbia Model)

Each client's treatment plan follows a structured Problem-Goal-Intervention model:

Step 1: Problem Identification (DSM-5 aligned)

- Substance Use Disorder
- Withdrawal Management
- Co-occurring Mental Health
- Relapse Risk
- Family/Relationship Dysfunction
- Housing Instability
- Employment/Vocational Deficit
- Spiritual Disconnection
- Trauma
- Legal Complications

Step 2: SMART Goals (Specific, Measurable, Attainable, Relevant, Time-bound)

- 3-7 goals per identified problem
- Goals co-authored with client
- Reviewed at MDT (Week 4, Week 8, Pre-discharge)

Step 3: Interventions (linked to goals)

- Individual CBT, Group CBT, MI, Psychoeducation, Spiritual Formation, Vocational Training, Family Therapy, Pastoral Counselling

Step 4: Progress Tracking

- Goal status: Not Started / In Progress / Completed / Deferred / Modified
- Treatment plan updated at each MDT review
- Client participates in all reviews

6.2 Cognitive Behavioural Therapy (CBT)

- **Individual:** 60-minute sessions, weekly minimum per client
- **Group:** 90-minute sessions, 3x weekly, max 20 participants
- **Focus:** Thought records, cognitive restructuring, HALT model (Hungry, Angry, Lonely, Tired), trigger mapping, coping strategy toolbox, refusal skills training
- **PRPP Development:** Personal Relapse Prevention Plan drafted in Phase 3, finalised in Phase 4

6.3 Motivational Interviewing (MI)

Applied throughout the programme, particularly during:

- Pre-admission screening
- Early engagement (Phase 1-2)
- Resistance or ambivalence about change
- Outpatient Engagement Pathway

Core MI techniques: Open questions, Affirmations, Reflective listening, Summaries (OARS)

6.4 Personal Relapse Prevention Plan (PRPP)

Every client must develop a written PRPP as a graduation requirement:

Trigger Identification across 5 categories:

1. People (former using friends, dealers, romantic partner)
2. Places (bars, former neighbourhood, certain routes)
3. Feelings (anger, loneliness, boredom, anxiety, shame)
4. Situations (financial pressure, arguments, celebrations, payday)
5. Times (evenings, weekends, holidays, paydays)

Minimum 5 Active Coping Strategies (from approved list):

- Deep breathing exercises
- Call sponsor/accountability partner

- Attend NA/AA or church-based recovery group
- Physical exercise
- Prayer and meditation on Scripture
- Journaling
- Contact crisis helpline (09011277600)
- Remove self from triggering environment
- Grounding techniques (5-4-3-2-1)
- Engage in hobby or creative activity
- Talk to trusted family member
- Read Bible / devotional material

Named Recovery Support Network:

- Sponsor/accountability partner
- Primary Family Support Person (Pathway A) or Social Worker (Pathway B)
- Church pastor or Life Centre leader
- NA/AA home group or recovery fellowship

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7. SPIRITUAL FORMATION PROTOCOL

7.1 Foundation

House of Refuge is not a clinical programme with a religious add-on. It is a Christ-centred home of healing, within which clinical care is delivered as a sacred act of service. The atmosphere must tangibly reflect the person and presence of Jesus Christ.

7.2 Daily Spiritual Programme

Element	Frequency	Description
Morning Prayer & Devotions	Daily, 7:00 AM	Corporate praise, worship, prayer, Scripture declaration
Bible School / Spiritual Formation	Mon-Fri, 9:00-11:00 AM	Structured, progressive curriculum from salvation through character formation
Evening Chapel / Bible Study	Daily, 7:00 PM	Word, prayer, testimony, peer spiritual accountability
Sunday Chapel Service	Weekly	Extended service with praise, worship, preaching, altar call, testimonies, prayer

Personal Devotions	Daily (scheduled time)	Individual prayer and Bible reading time
Peer Recovery Circle	Daily (informal, evening)	Peer accountability, encouragement, prayer

7.3 Spiritual Milestones

Tracked per client (status: Not Yet / In Progress / Achieved):

1. Accepted Jesus Christ as Lord and Saviour
2. Water baptism
3. Consistent daily personal devotion (prayer + Bible reading)
4. First public testimony shared
5. Regular participation in evening discipleship group
6. Serving others in the community ("as unto the Lord")
7. Demonstrating Fruit of the Spirit (Galatians 5:22-23)
8. Active intercessory prayer for others
9. Completed Bible school curriculum
10. Connected to local church for post-discharge

7.4 Fruit of the Spirit Assessment (Galatians 5:22-23)

Each of the 9 fruits assessed as: Not Evident / Emerging / Growing / Consistently Demonstrated

1. Love
2. Joy
3. Peace
4. Patience
5. Kindness
6. Goodness
7. Faithfulness
8. Gentleness
9. Self-Control

7.5 Church Placement Pipeline

Every graduating client must be connected to a local church:

1. Church identified (by Week 8)
2. Pastor contacted and briefed

3. First visit completed (during pass or Phase 4)
4. Integration plan established (small group, serving role)
5. Handover to church pastoral team at graduation

7.6 Christ-Centred Counselling Principles

Principle	Application
Compassion	Lead with compassion before correction (Mark 1:41)
Acceptance	Receive the client as Christ receives them - fully, before change (Luke 19)
Worth and Value	Help the client discover their infinite worth in God's sight (Mark 2:27)
Responsibility	Call to ownership of choices, lovingly not coercively (Luke 5:24)
Hope	The Gospel is the most powerful force of hope (Matt 11:28-30)
Cognitive Reframing	Redirect thinking from distorted to truth-aligned (Matt 19:4-9)
Confrontation with Truth	Compassionately confront denial and self-deception (John 8:3-11)
Confidence	All shared information kept strictly confidential (Prov 11:13)

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8. LIFE SKILLS CURRICULUM

8.1 Thirteen Group Modules

Module	Frequency	Facilitators	Focus
1. Morning Community Meeting	Daily (Mon-Fri), 8:30 AM	House Master + Counsellor	Accountability, day preview, community dynamics
2. Care Planning Group	Thursday, 12:00 PM	2 Clinical/Nursing Staff	Treatment plan participation, SMART goals, problem-solving
3. CBT and Relapse Prevention	3x weekly	Clinical Lead / Counsellor	Cognitive restructuring, trigger mapping, PRPP

4. Small Group (Process Group)	Wednesday, 12:00 PM	1-2 Facilitators	Feelings, relational patterns, assertiveness
5. Psychoeducation	2x weekly	Nurse / Counsellor	Addiction neuroscience, mental health, medication
6. Relaxation & Stress Management	Friday, 2:30 PM	Nurse + Counsellor	Deep breathing, PMR, guided visualisation (Biblical), stretching
7. Relationship and Family Group	Friday, 3:00 PM	1-2 Staff	Communication, conflict resolution, forgiveness, boundaries
8. Transition Group (Discharge Readiness)	Weeks 10-12, Thu/Fri	2 Staff	Reintegration fears, community resources, relapse risk
9. Work, Craft, and Vocational	Monday, 2:00 PM	2 Staff + Volunteers	Carpentry, crafts, gardening, tailoring basics
10. Cooking, Nutrition & Life Practical	Wednesday, 2:00 PM	Cook + Counsellor	Meal planning, budgeting, home safety, first aid
11. News and Current Affairs	Monday, 11:00 AM	2 Staff	News literacy, critical thinking, public speaking
12. Creative Writing	Thursday, 12:00 PM	2 Staff	Self-expression, insight, cognitive skills
13. End-of-Day Wrap-Up	Daily (Mon-Fri), 8:30 PM	House Master / Counsellor	Day review, reflection, accountability, prayer

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9. MEDICATION MANAGEMENT PROTOCOL

9.1 Standards

- All medications administered by licensed nursing staff ONLY
- Documented in Medication Administration Record (MAR)
- Nursing medication rounds: 6:00 AM, 12:00 PM, 6:00 PM, 10:30 PM
- **No opioid-containing medications** in the facility under any circumstances
- Medication storage in locked cabinet with controlled access
- Weekly stock reconciliation

- Expiry date monitoring (flagged at 90 days)

9.2 Approved Medication Categories

Category	Examples
Detox Support	Diazepam, Chlordiazepoxide, Lorazepam
Vitamin Supplement	Thiamine, Multivitamin, B-Complex, Folic Acid, Ferrous Sulphate
Psychiatric (non-opioid)	Haloperidol, Chlorpromazine, Risperidone, Amitriptyline, Fluoxetine
Anticonvulsant	Carbamazepine
Antihypertensive	As prescribed
Antiemetic	Metoclopramide, Promethazine
Analgesic (non-opioid)	Paracetamol, Ibuprofen
Antimalarial	Artemether-Lumefantrine
Antibiotic	Ciprofloxacin, Amoxicillin

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10. RISK ASSESSMENT & SAFETY PROTOCOL

10.1 Routine Risk Screening

Instrument	Frequency	Responsible
C-SSRS (Suicide Risk)	Admission, weekly, any clinical concern	Clinical staff
Psychosis screening	Admission, daily during detox, any concern	Nursing + Clinical
Violence risk	Admission, ongoing observation	All staff
Self-harm assessment	Admission, weekly during Phases 1-2	Nursing + Clinical

10.2 Drug-Induced Psychosis (DIP) Response Protocol

1. STAY CALM - Do not argue with delusions
2. ENSURE SAFETY - of client, other residents, and staff
3. NOTIFY nursing staff and Program Director IMMEDIATELY
4. PROVIDE continuous calm 1-to-1 supervision

5. Do NOT leave client unattended until medical assessment complete
6. Program Director initiates IMMEDIATE REFERRAL to Yaba Psychiatric Hospital
7. FAMILY NOTIFICATION
8. DOCUMENT FULLY in client file

10.3 Emergency Escalation Pathway

Immediate safety risk → Nurse/Crisis Officer → Clinical Lead → Program Director → Board (if serious) Safeguarding concern → Designated Safeguarding Lead / Program Director immediately

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11. FAMILY ENGAGEMENT PROTOCOL

11.1 Pathway A (Family-Supported)

Requirement	Timing	Standard
Family psychoeducation	Within first 2 weeks	Understanding addiction as a brain disorder
Family therapy session 1	By Week 4	Initial assessment, expectations, communication
Family therapy session 2	By Week 8	Boundaries, enabling behaviour, recovery roles
Pre-discharge family meeting	Week 11	Reintegration plan, roles, boundaries, aftercare
Home environment assessment	By Week 10	Safety and sobriety-supportiveness

11.2 Pathway B (Community-Supported)

Requirement	Timing	Standard
VNA completion	At admission	Vulnerability and Needs Assessment
Social Worker case management	Throughout + 12 months post	Primary case manager role

Church/NGO partner engagement	By Week 4	Community support ecosystem
Supported housing referral	By Week 8	Transitional or permanent placement
Vocational pathway	Before discharge	Training or employment identified

11.3 Family Visitation Rules

- **When:** Sunday, 12:00 PM - 6:00 PM (maximum 6 hours)
- **Frequency:** Once per week maximum
- **Location:** Designated visitor areas only. No dormitory access
- **Children:** Must be supervised by parent/guardian at all times. Never alone with any resident
- **Friend visits:** Require Program Officer approval in advance
- **Unscheduled visits:** Not permitted without approval

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12. BEHAVIOURAL MANAGEMENT PROTOCOL

12.1 Philosophy

Discipline at HOR is one of the most powerful therapeutic tools available. It teaches self-governance, delayed gratification, and accountability. It is NEVER punishment or humiliation.

> *"No discipline seems pleasant at the time, but painful. Later on, however, it produces a harvest of righteousness and peace for those who have been trained by it." - Hebrews 12:11*

12.2 Three-Tier System

Tier 1 - Minor Offenses:

- Examples: Poor hygiene, dormitory cleanliness, lateness, food in rooms, phone abuse, laziness on duties

- Responses: Verbal warning (documented), written reflection exercise, temporary privilege loss

Tier 2 - Major Offenses:

- Examples: Substance use on pass, smoking on premises, stealing, property damage, threats, not returning from pass
- Responses: Formal reprimand, extra work assignments, written confession/apology, Bible passage memorisation, loss of all privileges, motivational discipline activated

Tier 3 - Motivational Discipline:

- Applied for: Disengagement pattern, spiritual stagnation, consistent failure to progress
- Duration: Minimum 2 weeks, reviewed weekly by Program Director
- Measures: Special work crew, graduation date on hold, loss of all pass/phone, all free time redirected to prayer/Bible study/reflection, weekly evaluation meetings

12.3 Dismissal Offenses

- Being drunk or under the influence on premises
- Leaving facility without permission
- Physical assault
- Persistent excessive rule violations
- Refusal to accept disciplinary response
- Sexual misconduct
- Causing severe disunity

12.4 Dismissal Procedure

1. Program Director consults Chaplain, informs Executive Director and Board
2. Full documentation of violations
3. Compassionate private conversation explaining reasons
4. Time to pack belongings
5. Contact family for transportation
6. Prayer offered. Referrals provided
7. Transport arranged if needed
8. **NO CLIENT LEAVES WITHOUT A SAFETY PLAN**

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13. DISCHARGE & GRADUATION PROTOCOL

13.1 Four Discharge Types

Type	Description	Protocol
Type 1: Planned (Graduation)	Completes programme, meets all 6 criteria	Full graduation ceremony
Type 2: Administrative	Dismissed for serious behavioural violation	30-day minimum before re-admission considered
Type 3: Clinical	Needs exceed HOR scope (psychiatric/medical)	Written referral, accompanied transport
Type 4: Self-Discharge (AMA)	Voluntary departure before completion	AMA form signed, harm reduction counselling

13.2 Six Graduation Criteria (ALL Must Be Demonstrated)

#	Criterion	Standard	Assessment Source
1	**Clinical Stability**	No withdrawal, stable vitals, no psychiatric symptoms, sustained abstinence	Nursing records + Doctor sign-off
2	**Behavioural Compliance**	Rule compliance last 4 weeks, positive community participation	Counsellor report
3	**Psychological Readiness**	Clear insight into addiction, articulates PRPP verbally and written	Counsellor + PRPP review
4	**Relapse Prevention Competence**	Written PRPP complete, 5+ coping strategies, named support network	PRPP document
5	**Spiritual Formation**	Authentic Christian growth, values transformation (not mere compliance)	Chaplain report
6	**Reintegration Readiness**	Confirmed safe placement, family/community meeting done, aftercare finalised	Social Worker / Counsellor

13.3 Non-Negotiable Discharge Standard

Regardless of discharge type, EVERY client leaves with:

- Basic safety plan (emergency contacts, crisis line, nearest health facility)
- Harm reduction information (if self-discharging or administrative)
- Written referral to appropriate community services
- HOR's contact number
- Prayer and a compassionate farewell

13.4 Re-Admission Policy

HOR operates an open-door re-admission policy. Relapse is a clinical event, not a moral failure.

Criterion	Standard
Willingness re-established	Full assessment conducted
Relapse reflection	Articulate what contributed and what will differ
Exclusion criteria absent	All original criteria still apply
Administrative discharge gap	Minimum 30 days; demonstrate changed attitude
Modified treatment plan	New individualised plan, not a repeat
Re-admission interview	Conducted by Program Director

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14. AFTERCARE & ALUMNI PROTOCOL

14.1 Post-Discharge Monitoring (24 Months)

Period	Contact Frequency	Home/Workplace Visit
Months 1-3	Weekly phone contact	Month 3
Months 4-6	Bi-weekly phone contact	Month 6
Months 7-12	Monthly phone contact	Month 12
Year 2	Monthly phone contact	Bi-monthly
Month 24	Case closure assessment	Final visit

14.2 Outcome Snapshots (at 3, 6, 12, 24 months)

Structured assessment at each milestone:

- Sobriety status (Abstinent / Lapse / Relapse / Unknown)
- Employment status (Employed / Self-employed / Seeking / In training)
- Church attendance (Weekly / Bi-weekly / Monthly / Occasionally / Not attending)
- Family status (Reunified / In progress / Estranged)
- Housing (Stable / Transitional / Unstable / Homeless)
- Support group engagement (Active / Occasional / None)
- Overall assessment (Thriving / Stable / At risk / Relapsed / Lost to follow-up)

14.3 Relapse Response Protocol

If relapse detected at any monitoring point:

1. Immediate compassionate outreach
2. Risk assessment conducted
3. Outreach counselling offered
4. Residential re-admission offered (if willing)
5. Harm reduction information provided
6. Family/support network engaged (with consent)

14.4 Alumni Programme Components

Component	Description
Alumni Support Groups	Regular gatherings for peer accountability and encouragement
Mentorship Programme	Stabilised alumni (6+ months) trained as recovery mentors
Programme Participation	Testimonies at chapel, speaking to current residents
Continued Counselling	Access to HOR counsellors for periodic sessions
Church Integration	Connection to Life Centres and local church fellowships

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15. CLINICAL OUTCOME MEASUREMENT

15.1 Validated Instruments (Admission vs Discharge)

Instrument	Domain	Score Range	Interpretation

AUDIT	Alcohol severity	0-40	0-7 low, 8-15 hazardous, 16-19 harmful, 20+ dependence
DAST-10	Drug severity	0-10	0 none, 1-2 low, 3-5 moderate, 6-8 substantial, 9-10 severe
PHQ-9	Depression	0-27	0-4 minimal, 5-9 mild, 10-14 moderate, 15-19 moderately severe, 20-27 severe
GAD-7	Anxiety	0-21	0-4 minimal, 5-9 mild, 10-14 moderate, 15-21 severe

15.2 Programme KPIs (Reported Quarterly to Board)

KPI	Target	Measurement
Graduation rate	>75%	Completed / Admitted
12-month abstinence rate	>60%	Alumni monitoring data
Employment at 6 months	>50%	Outcome snapshots
Church attendance at 6 months	>70%	Outcome snapshots
Family reunification rate	>60%	Outcome snapshots
Re-admission rate (positive)	Track	Re-admission / Total graduates
Adverse events	Zero target	Incident reports

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16. QUALITY ASSURANCE & GOVERNANCE

16.1 Governance Structure

Level	Role
Board of Trustees (Freedom Foundation)	Strategic direction, policy approval, financial oversight
Executive Committee	Operational oversight, resource mobilisation
Executive Director	Day-to-day operations, human capital, financial stewardship

HOR Governing Board	Programme curriculum, client welfare, staff conduct, policy adherence
Program Director (Medical Doctor)	Clinical and operational leader of HOR

16.2 Multi-Disciplinary Team (MDT)

Composition: Program Director (chair), Head of Clinical Services, Head Nurse, Social Worker, House Master, Chaplain, Clinical Psychologist

Frequency: Weekly (standard), emergency meetings for critical incidents

Functions: Care plan reviews, relapse risk identification, discharge planning coordination, care delivery problem-solving

16.3 Staff Requirements

Role	Qualification	Registration
Program Director	Medical Doctor, addiction medicine experience	Medical council
Clinical Lead	Degree in psychology/counselling + CBT/MI training	Professional registration
Nursing Staff (3 rotating)	Registered Nurse	Nursing council
Social Worker	Degree in social work	Social work registration
Chaplain	Pastoral training + counselling skills	Church ordination/endorsement
House Master	Character, lived experience acceptable	Safeguarding-trained

16.4 Safeguarding Standards

- All staff complete mandatory safeguarding training within 30 days of hire
- Annual safeguarding refresher training
- Zero-tolerance for abuse, neglect, exploitation
- Professional boundaries: no dual relationships between staff and clients
- Designated Safeguarding Lead appointed
- All allegations investigated immediately and reported to Board
- Whistleblowing protection in place

16.5 Protocol Review

This Treatment Protocol is reviewed:

- Annually by the Program Director and Governing Board
- When required by changes in evidence, regulations, or programme learning
- After any serious incident or adverse event

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17. APPENDICES

Appendix A: Consent Forms Checklist

1. Admission Agreement
2. Informed Consent for Treatment
3. Detoxification Consent
4. Confidentiality Agreement
5. Family Confidentiality Agreement (Pathway A)
6. Belongings Receipt
7. Financial Agreement
8. Rights and Responsibilities Charter

Appendix B: Clinical Assessment Schedule

Assessment	Admission	Weekly	Fortnightly	Monthly	Discharge
C-SSRS	X	X			X
CIWA-Ar (alcohol)	X	As needed			
COWS (opioid)	X	As needed			
AUDIT	X				X
DAST-10	X				X
PHQ-9	X			X	X
GAD-7	X			X	X
URICA	X		X		X

Vitals (BP, Pulse, Temp)	X				X
UDS	X			X	X
Weight / BMI	X			X	X

Appendix C: Referral Partner Directory

Partner	Services
Yaba Psychiatric Hospital	Psychiatric assessment, inpatient care, prescription management
Rights and Humanity	Legal advice and assistance
NDLEA	Regulatory liaison, national drug treatment standards
Lagos State Ministry of Health	Regulatory oversight, HIV/ART referral
Freedom Foundation Empowerment Unit	Vocational training, seed capital, employment
Freedom Foundation Educational Unit	Further education opportunities
Life Centres (This Present House)	Church placement, pastoral mentoring, fellowship
Private Psychiatric Providers	Confidential private psychiatric care
HIV/ART Services	Government or private ART facility linkage

Document End

House of Refuge Rehabilitation Centre | A Freedom Foundation Programme | 2026 Edition

"For freedom Christ has set us free." - Galatians 5:1

Prepared by: Programme Development Team **Clinical Review:** Program Director (Medical Doctor) **Spiritual Review:** Chaplain / Spiritual Formation Lead **Approved by:** HOR Governing Board **Date:** April 2026